



This factsheet is about having surgery to reverse your stoma. It explains what a stoma reversal is, possible complications and what to expect after the operation.

What is a stoma?

A stoma is where a section of bowel is brought out through an opening on your stomach area (abdomen). Your bowel movements (poo) are collected in a pouch or bag attached to the skin around your stoma. A stoma can be temporary or permanent.

A temporary stoma is formed to allow the bowel to heal after surgery. Once the bowel has healed, a temporary stoma can usually be reversed.

Some people can't have a stoma reversal and the stoma will be permanent. You can read more about this on page 2.

What is a stoma reversal?

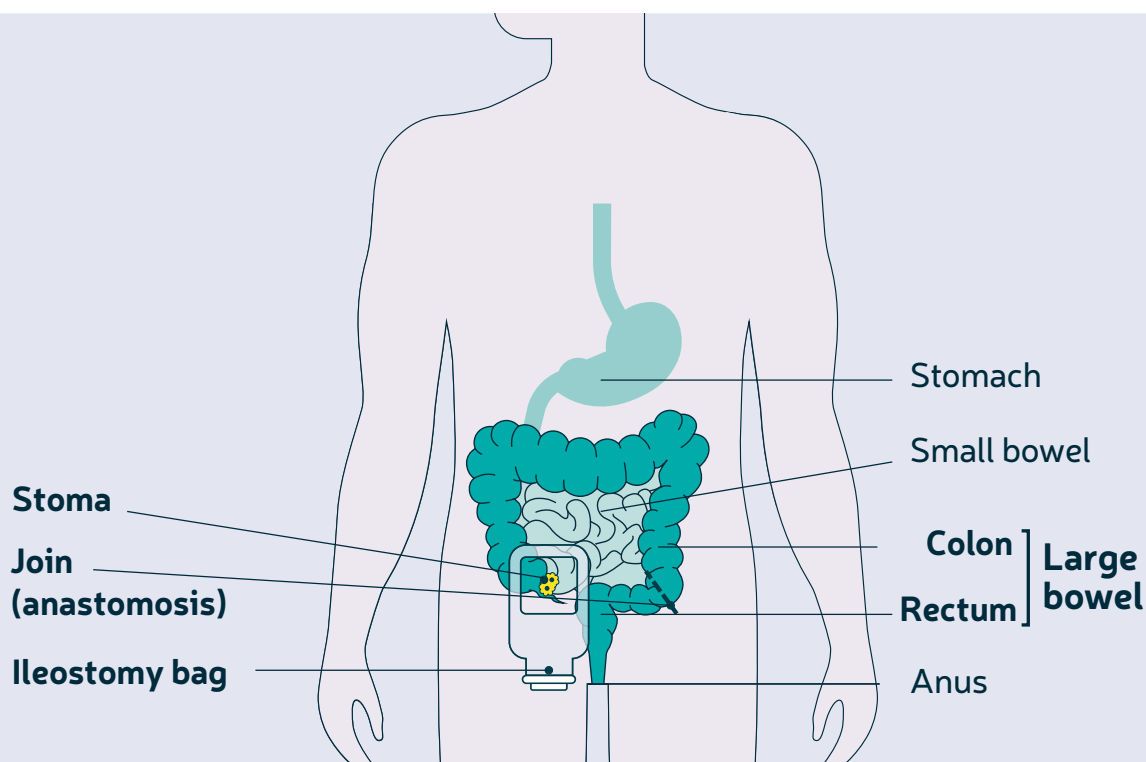
Stoma reversal surgery involves re-joining the two healthy ends of your bowel and closing the stoma that was formed during your first operation. It means you'll no longer need your stoma bag and you'll empty your bowels by going to the toilet.

Ileostomy

An ileostomy is a stoma formed by bringing the end or a loop of the small bowel (ileum) onto the surface of the stomach area.

Colostomy

A colostomy is a stoma formed by bringing part of the large bowel (colon and rectum) onto the surface of the stomach area.



About stoma reversal

Who can have a stoma reversal?

Not everyone can have their stoma reversed. Your surgeon will talk to you about whether a reversal is the right option for you.

They will want to make sure that:

- the muscles in your anus are working. If your cancer removal surgery damaged the anal sphincter muscles, you may struggle to control your bowel movements and could have some leakage from your bottom
- you have enough bowel left and it has healed
- you're fit and healthy enough for more surgery

When will you have your stoma reversed?

It's possible to have a stoma reversal two to four months after your stoma was formed. But it's more likely that you'll wait much longer than this. You may need to wait for any swelling to go down or to recover from other treatment, such as chemotherapy. There may also be a waiting list for surgery.

In some cases, people can have a stoma for several years before having a reversal operation. Contact your healthcare team if you don't receive a date for your operation.

Before your stoma reversal

Your healthcare team will ask you to sign a form to agree (consent) to the stoma reversal operation. Make sure you understand what surgery involves and that you've had a chance to ask questions.

Your specialist nurse will give you information on how to get ready for your operation. This will include advice on what to eat and drink and what to take into hospital with you.

They may also give you information on pelvic floor exercises. Your pelvic floor muscles support your bladder and rectum. When you have a stoma, you don't use these muscles so they can get weaker. This can make it harder to control bowel movements after your stoma has been reversed. Doing pelvic floor exercises for a few months before your reversal operation can strengthen these muscles and help your recovery after surgery. Ask your stoma care nurse for more information.

Before your operation, the surgeon will check that your bowel has healed. They may do this by:

- using a gloved finger to check your rectum – they may also use a camera to see where the bowel has been joined back together
- giving you a liquid (enema) through your bottom into your bowel – the liquid shows up on X-ray so the doctor can see any leaks around the join

About stoma reversal

Your stoma reversal surgery

The type of stoma reversal you have will depend on the surgery you had when your stoma was formed.

To reverse a loop colostomy or ileostomy, the surgeon will make an opening around the existing stoma. They'll then join the ends of the bowel together. Sometimes, your surgeon will need to open part of the scar from your previous operation.

If you had an end colostomy or ileostomy, the surgeon will make a larger opening in the stomach area (abdomen) before joining the two ends of bowel together.

Your surgeon will talk to you about what your surgery will involve.

After your stoma reversal surgery

You'll usually be able to leave hospital three to ten days after your operation. This will depend on what type of surgery you had and how well you recover. You can usually go home once you're eating and drinking, have passed wind and are able to move around without too much discomfort.

While you're in hospital, the nurses will check your wound each day and tell you how to look after your wound at home.

Possible risks of stoma reversal surgery

Many people don't have serious problems after having stoma reversal surgery.

Your surgeon will explain the possible risks, which may include:

- **wound infection** – if your wound gets infected, you might need antibiotics. Tell your healthcare team as soon as possible if your wound is hot, red or sore
- **ileus** – after surgery, your bowel might be slow to start working again or stop working. This is a condition called ileus. You may feel bloated, have pain in your stomach area and feel or be sick. You'll need to stop eating and drinking until you start to pass wind again
- **anastomotic leak** – if the newly joined ends (anastomosis) of the bowel don't heal properly, the bowel contents can leak into the abdomen. This can cause pain in your lower tummy area, a high temperature and tiredness. You may need to rest your bowel by not eating or drinking for a few days. Or you may need more surgery and possibly another stoma
- **bowel obstruction** – sometimes, the bowel can become blocked. If this happens, you may need to rest the bowel by not eating or drinking, or you may need another operation

Contact your healthcare team if you feel unwell or have any symptoms after your surgery.

About stoma reversal

Recovery at home

To help your recovery at home:

- **Avoid heavy lifting or bending for six weeks after your operation** – you may want to plan any changes at work or support at home in advance
- **Check with your doctor and insurance company when you can start driving again and start some gentle exercise, such as walking, soon after your operation** – you could start with a walk around the house then move on to a short walk outside. As you get your strength and energy back, you'll be able to do more
- **Protect your skin** – you may want to use moist toilet wipes and barrier creams, such as nappy rash creams, to help stop your bottom getting sore
- **Listen to your body and rest when you need to** – it can take a few weeks to get your energy back

Regaining bowel control after stoma reversal

Your bowel is likely to work differently from before you became ill because there's a piece of your bowel missing. There will be parts of your bowel that won't have been working while you had your stoma. This can lead to changes in your bowel patterns after stoma reversal surgery, which take time to get back to normal. This will vary from person to person.

You may have loose, runny poo (diarrhoea) and you may need to empty your bowels more urgently and more often. This should settle over a few weeks or months as your bowel recovers.

Speak to your healthcare team about the best way to manage your symptoms.

They may suggest you take medicines or make some changes to your diet and lifestyle.

Low anterior resection syndrome

If you had surgery for rectal cancer, you may have long-term problems after stoma reversal surgery. This is called low anterior resection syndrome (LARS).

Symptoms can include:

- needing to empty your bowels urgently
- feeling that you have not fully emptied your bowels
- having small, more frequent bowel movements
- difficulty knowing whether you need to pass wind or a bowel movement
- not being able to control when you have a bowel movement

Contact your GP or specialist nurse if you have any of these symptoms. They can give you medicine or refer you for further investigation and treatment.

Pelvic floor exercises may also help to improve symptoms. These strengthen the muscles around your rectum and anus. We have more information about muscle training and pelvic floor exercises in our booklet, 'Regaining bowel control'.

About stoma reversal

Your feelings and emotions

It's natural to feel embarrassed, worried or helpless at times after bowel surgery. But it's important to speak to your healthcare team about any worries or bowel problems so you can get treatment and support.

If you're having problems coping or feel you need some extra support, your GP can refer you to a counsellor.

Your sex life

You may not feel like having sex for a while after your operation. If you're in a relationship, try to keep talking about how you're both feeling and find other ways to be close.

Once you're feeling stronger, you may feel able to start having sex again. If you have any problems, speak to your GP or healthcare team.

Managing your diet

After your stoma reversal surgery, your bowel is likely to be sensitive. To help with this, you could:

- eat little and often, rather than having large meals
- avoid eating late at night
- drink plenty of fluids, including some water

After your stoma reversal it may be tempting to eat all the foods you've missed, but some foods can irritate your bowel. Try to limit or avoid:

- citrus fruits such as oranges, grapefruits and lemons
- spicy food such as curries and foods containing chillies
- high-fibre foods, such as fruit, vegetables and wholegrains
- foods high in fat such as cheese, eggs, avocado and salmon
- too much alcohol, caffeinated tea and coffee or fizzy drinks

You can gradually add these foods to your diet once your bowel has settled down. It might take a bit of trial and error to find foods that work for you. You can read more about diet in our 'Eating well' booklet.